**CRYPTORCHISM** The failure of one or both testes to descend down into the scrotal sacs is known as cryptorchism (crypto – hidden + orchis – testicle). It occurs in 1 – 3 percent of new born males. A surgical correction at a young age can rectify the defect, else these individuals may become sterile and are unable to produce viable sperms.

androgens namely the testosterone hormone which initiates the process of spermatogenesis. These cells are endocrine in nature and are characteristic features of the testes of mammals. Other immunologically competent cells are also present.

The accessory ducts associated with the male reproductive system include **rete testis, vasa efferentia, epididymis and vas deferens (Fig. 2.2 b)**. The seminiferous tubules of each lobule converge to form a tubulus rectus that conveys the sperms into the rete testis. The rete testis is a tubular network on the posterior side of the testis. The sperms leave the rete testis and enter the epididymis through the vasa efferentia. The epididymis is a single highly coiled tube that temporarily stores the spermatozoa and they undergo physiological maturation and acquire increased motility and fertilizing capacity. The epididymis leads to the vas deferens and joins the duct of the seminal vesicle to form the ejaculatory duct which passes through the prostate and opens into the urethra. The urethra is the terminal portion of the male reproductive system and is used to convey both urine and semen at different times. It originates from the urinary bladder and extends through the penis by an external opening called urethral meatus.

The accessory glands of the male reproductive system include the paired **seminal vesicles** and **bulbourethral glands** also called **Cowper's gland** and a single **prostate gland**. The seminal vesicles secrete an alkaline fluid called seminal plasma containing fructose sugar, ascorbic acid, prostaglandins and

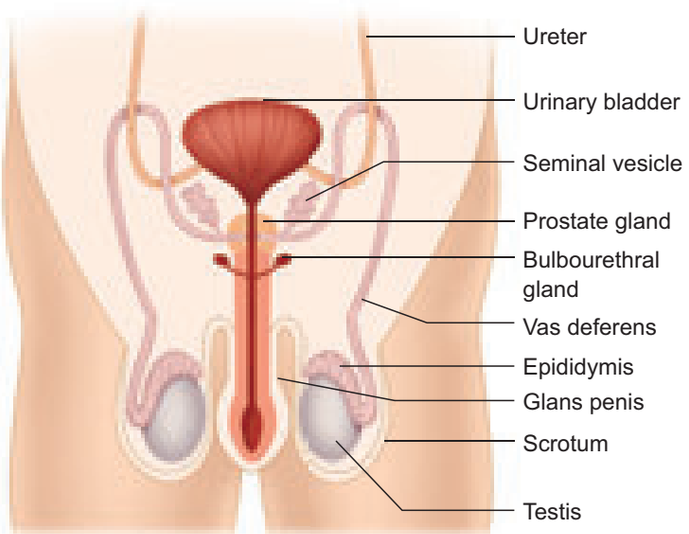



**Fig. 2.2 (a) Diagrammatic view of the male reproductive system**

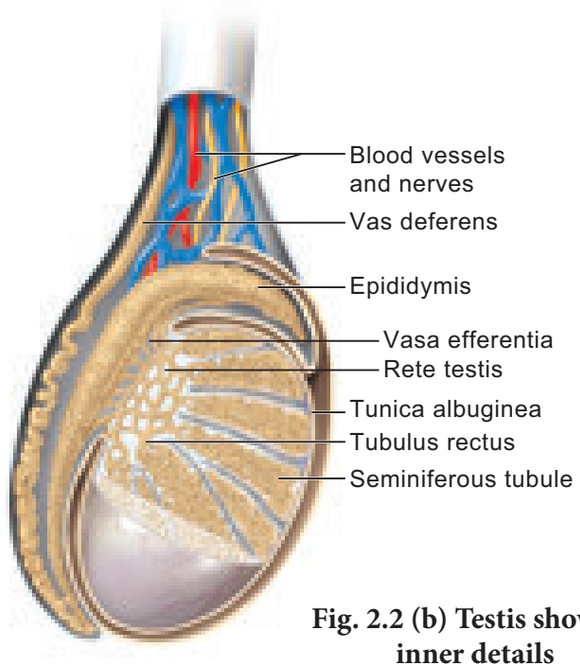



**Fig. 2.2 (b) Testis showing inner details**



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